Table 1. Classification and management of blood pressure for adults*

BP Classification	SBP* mmHg	DBP* mmHg	Lifestyle Modification	Initial drug therapy	
				Without Compelling Indication	With Compelling Indications (See Table 8)
Normal	<120	and <80	Encourage		
Prehypertension	120–139	or 80–89	Yes	No antihypertensive drug indicated.	Drug(s) for compelling indications.‡
Stage 1 Hypertension	140–159	or 90–99	Yes	Thiazide-type diuretics for most. May consider ACEI, ARB, BB, CCB, or combination.	Drug(s) for the compelling indications.‡ Other antihypertensive drugs (diuretics, ACEI, ARB, BB, CCB) as needed.
Stage 2 Hypertension	≥160	or ≥100	Yes	Two-drug combination for most† (usually thiazide-type diuretic and ACEI or ARB or BB or CCB).	

DBP, diastolic blood pressure; SBP, systolic blood pressure.

Drug abbreviations: ACEI, angiotensin converting enzyme inhibitor; ARB, angiotensin receptor blocker; BB, beta-blocker; CCB, calcium channel blocker.

* Treatment determined by highest BP category.

† Initial combined therapy should be used cautiously in those at risk for orthostatic hypotension.

‡ Treat patients with chronic kidney disease or diabetes to BP goal of <130/80 mmHg.

als 40–70 years of age, each increment of 20 mmHg in systolic BP (SBP) or 10 mmHg in diastolic BP (DBP) doubles the risk of CVD across the entire BP range from 115/75 to 185/115 mmHg.[8]

The classification "prehypertension," introduced in this report (table 1), recognizes this relationship and signals the need for increased education of health care professionals and the public to reduce BP levels and prevent the development of hypertension in the general population.[9] Hypertension prevention strategies are available to achieve this goal. (See "Lifestyle Modifications" section.)

BENEFITS OF LOWERING BLOOD PRESSURE

In clinical trials, antihypertensive therapy has been associated with reductions in stroke incidence averaging 35–40 percent; myocardial infarction, 20–25 percent; and heart failure, more than 50 percent.[10] It is estimated that in patients with stage 1 hypertension (SBP 140–159 mmHg and/or DBP 90–99 mmHg) and additional cardiovascular risk factors, achieving a sustained 12 mmHg reduction in SBP over 10 years will prevent 1 death for every 11 patients treated. In the presence of CVD or target organ damage, only 9 patients would require such BP reduction to prevent a death.[11]